DRAFT FOR DISCUSSION

Delhi Precision Medicine Center

Qutub Institutional Area, Mehrauli, South Delhi

Prototype 1 of the VPS Lakeshore network — haemato-oncology, organ transplantation, rare disease, precision oncology and genomics

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DELHI NCR

Precision Medicine Market & Rare Disease Landscape

India's nucleus for precision medicine, genomics innovation and rare disease care

PRECISION MEDICINE MARKET

₹4,200 Cr
Delhi NCR Precision Medicine Market (2024)

18.4%
Projected CAGR 2024–2030

62+
Genomics & Oncology Diagnostic Labs in NCR

~4.2L
New cancer cases NCR / year

#1
Lung, Breast & Cervical Ca burden

RARE DISEASE LANDSCAPE

8,000+
Rare diseases identified globally

7.2 Cr
Rare disease patients in India

12–15%
Estimated NCR share of India rare disease burden

₹580 Cr
Rare disease diagnostics market, NCR 2024

NCR combines scale, clinical depth and referral gravity—making it the most investible precision medicine cluster in India.

NCR GEOPOLITICAL DISTRICT MAP

HARYANA
Sonipat • Rohtak • Jhajjar
Gurugram • Nuh • Faridabad • Palwal

UTTAR PRADESH
Baghpat • Ghaziabad
GB Nagar (Noida)

NCT DELHI

Highest concentration of genomics labs & specialized diagnostic centers

Strong network of multi-specialty hospitals & oncology centers

Large patient pool & referral gravity across North India

Growing research ecosystem and innovation in precision medicine

Sources: ICMR, MOHFW, PharmaBio India 2024, FICCI Health Report, National Rare Diseases Policy 2021, Census India, geoBoundaries (CC-BY)

Data for strategic planning purposes

Delhi NCR — India's deepest premium care catchment, with a documented demand-supply gap in quaternary and precision-led services.

₹77k	74%	26.8%	19.7%
Q4 FY25 ARPOB Max Healthcare	FY25 occupancy Max Healthcare	FY25 EBITDA margin Max network	FY24 EBITDA margin Narayana Health India

Demand-supply gap

Max Healthcare and Apollo both identify unmet quaternary demand in Delhi NCR. The city's concentration of insured, NRI and high-income households makes it the strongest catchment in India for precision-led pricing. Competitor operators explicitly note the gap for complex blood disorders, transplantation and rare disease.

Benchmark operating economics

Max FY25 EBITDA at 26.8% and Narayana India at 19.7% demonstrate what disciplined specialty mix delivers at scale. Delhi PMC does not require heroic assumptions — it requires a disciplined specialty stack and a managed ramp.

Site and catchment

Qutub Institutional Area is embedded in South Delhi's institutional corridor — IIT Delhi ~1 km north, JNU ~1.5 km west. Metro access at Qutub Minar (~800 m) and Malviya Nagar (~1.2 km). The site positions Delhi PMC within the highest GP-density catchment in NCR.

Sources: Max Healthcare Q4 FY25 earnings update · Max investor presentation Aug 2025 · Apollo Q4 FY25 results · Narayana Health Annual Report FY24

Five interdependent clinical programs that anchor the hospital

I — Haemato-Oncology, BMT & CAR-T

Adult and paediatric HSCT · allo/auto transplantation · CAR-T cellular therapy · GVHD management · acute leukaemias · MDS/MPN · lymphomas · haemophilia

Long LOS · BMT and CAR-T · continuous infusion · ICU-dependent · highest revenue per episode

II — Organ-Specific Transplantation

Kidney (highest volume) · liver · multi-organ · transplant hepatology · hepatobiliary surgery · dedicated transplant ICU · heart/lung optional

High ticket size · full utilisation of OT, ICU and dialysis assets · dedicated programme identity

III — Rare Disease, Metabolic & Immunology

Lysosomal storage disorders · SMA/muscular dystrophy gene therapy · primary immunodeficiency · ERT/IVIG/factor concentrate infusions · metabolic disorders

Lifelong patient cohort · predictable ERT/IVIG cycles · low surgery, high daycare · genetics dependency · national second-opinion franchise

IV — Precision Oncology — Solid Tumours

Rare cancers · sarcoma · neuro-oncology · complex resections · robotic-assisted surgery · molecular pathology · MDT-driven care · AYA oncology

Complex resections · high infusion · MDT-driven · medium and unpredictable LOS · tumour-board referral engine for NCR oncologists

V — Genomics, Prevention & Clinical Trials

Precision prevention · hereditary cancer screening · NGS panels · pharmacogenomics · Phase I–III trials · bioinformatics · germline variant testing · ICMR registry

Research revenue · feeder pathway to complex episodes · diagnostic differentiator · differentiates from generalist competitors

FULL STACK

G+6 structure with four basements. Governed by infection control and program logic

FLOOR	PROGRAMME ZONE	CLINICAL & FUNCTIONAL CONTENT
ROOF	Plant & Services	HVAC · MGPS/LMO/O_2 plant · Solar PV · ETP/STP/WTP/RO · IT riser · Fire suppression
F6	BMT & Cellular Therapy	8–10 HEPA+ positive-pressure rooms (allo/auto HSCT, CAR-T, GVHD) · Procedure Room (laminar flow) · BMT Day Care (6 bays) · Pharmacy Satellite · Digital Command Centre (dashboards, AI, telemedicine hub)
F5	Haemato-Oncology ICU + Isolation	ICU (10 beds, ventilated — neutropenic sepsis, CRS/ICANS, post-BMT) · HDU (8 beds, step-down) · AIIR Isolation (4 rooms, neg. pressure, independent exhaust) · Buffer floor — AHU entirely separate from F6
F4	Surgical OT Suite	Transplant OTs ×3 (liver/kidney; hepatobiliary; perfusion-capable) · Onco Surgery OTs ×2 (complex resections; sarcoma; robotic; laminar flow) · Hybrid OT ×1 (IR + vascular + image-guided) · Routine OTs ×2 · Pre/Post Op (12 bays) · CSSD satellite · Surgeon/anaesthetist offices
F3	Complex Care Wards — 74 Beds	Haematology & Blood Disorders Ward (25 beds) · Immunocompromised/High-Supervision Ward (15 beds, neg. pressure, ventilator-ready) · Complex Care Ward (20 beds — sarcoma, neuro-onco, AYA, neuromuscular) · Palliative & Symptom Management (8 beds) · Neuromuscular Zone (6 beds, ventilator-capable, ceiling hoist-ready) · Pharmacy Satellite
F2	Infusion, Day Care & Clinical Trials	Complex Infusion (20 bays — chemo, immunotherapy, bispecifics, trial protocols) · Rare Disease Infusion (12 bays — ERT, IVIG, factor concentrates) · Clinical Trials Unit (Phase I: 4 beds; Phase II/III: 10 bays; IMP pharmacy; data management) · MDT Boardrooms ×2 (video-linked) · Genomics & Tumour Board Support
F1	Specialist OPD — 30 Rooms	Rare Cancer · Haematology & Blood Disorders · Neuromuscular · Immunology & PID · Metabolic & Lysosomal · Oncogenetics · Genomic Counselling rooms (min. 4, soundproofed) · Clinical Nutrition · Nurse stations · Programme-differentiated waiting
GF	Reception, Prevention & Urgent Care	PMC Reception (atrium, triage, financial counselling, patient & family resource centre) · Precision Prevention Clinic (genomic risk, liquid biopsy, hereditary cancer screening — well-patient zone, aspirational aesthetic) · Oncology Urgent Care/Toxicity Review (4 bays, separate entry) · OPD Pharmacy · Dialysis (12 stations) · USG on-floor · Cafeteria/retail
B1	Parking & Staff Support	~300 car parking · Staff change rooms, canteen, lockers · HR/admin back-office · Biomedical engineering workshop · Security control room
B2	Imaging + CSSD + Central Pharmacy	Advanced Radiology (MRI ×2: 3T + 1.5T; PET-CT with dedicated lead-lined uptake rooms; CT ×1; SPECT-CT ×1; image-guided procedure room) · CSSD (full sterile processing for all OTs + BMT) · Central Pharmacy & Cold Chain (ISO Class 5 cleanroom; biologic/gene therapy storage; IMP vault; narcotics vault) · MDT Boardrooms ×2
B3	Laboratory, Genomics & Biobank	Central Lab (haematology, coagulation, biochemistry, surgical pathology, cytology, frozen section, IHC, FISH, cytogenetics, microbiology) · Advanced Blood Bank (24×7; Apheresis Unit 4 chairs) · Molecular Diagnostics & Genomics (NGS panels; pharmacogenomics; germline testing; bioinformatics) · Biobank & Research Repository (ICMR registry-linked) · Cancer Registry
B4	Engineering Plant	HVAC plant · Medical gas manifolds · Electrical substations, UPS, DG sets · IT data centre · Fire suppression systems · Biomedical waste holding · Back-of-house staff entry
LOWER B4	Radiation Oncology & Nuclear Medicine	LINAC ×2 (Vault 1: operational Day 1; Vault 2: structural shell, commission at ~40 Rx/day) · HDR Brachytherapy · CT Simulator · Dosimetry & Treatment Planning · Tumour Board Room · Nuclear Medicine (SPECT-CT; PET-CT; Hot Lab; HDT; injection rooms; radionuclide storage) · Wide-bore CT

136 licensed beds. 62 daycare and infusion bays. 8 operation theatres

INPATIENT BEDS	N
BMT Beds (HEPA+ positive-pressure)	**10**
Haemato-Oncology ICU (ventilated)	10
Step-Down / HDU	8
AIIR Isolation (negative-pressure)	4
ICU / Critical Care Subtotal	**32**
Haematology & Blood Disorders Ward	25
Immunocompromised / High-Supervision Ward	15
Complex Care Ward	20
Palliative & Symptom Management	8
Neuromuscular Zone (ventilator-capable)	6
Ward Subtotal	**74**
Pre & Post Operative	12
Endoscopy / DSA / RT Recovery	10
Oncology Urgent Care	4
Phase I Clinical Trial Suites	4
TOTAL LICENSED BEDS	**136**

DAY CARE & TREATMENT	N
Complex Infusion Bays (chemo, immunotherapy, bispecifics)	20
Rare Disease Infusion Bays (ERT, IVIG, factor concentrates)	12
Critical Infusion Beds (single cubicles)	6
Clinical Trials Bays (Phase II/III)	10
Dialysis Stations (transplant prep & bridging)	12
Apheresis (Blood Bank, B3)	4
DAYCARE / INFUSION TOTAL	**64**
OPERATION THEATRES (F4)	**N**
Transplant (liver/kidney; hepatobiliary; perfusion-capable)	3
Onco. Surgery (complex resections; sarcoma; robotic; laminar flow)	2
Hybrid (IR + vascular + image-guided)	1
Routine (day cases, port insertions, EUA)	2
TOTAL OTs	**8**
SPECIALIST PROCEDURE SPACES	N
LINAC vaults (Vault 1 operational + Vault 2 structural shell)	×2
HDR Brachytherapy suite	×1
CT Simulator (radiation oncology)	×1
BMT Procedure Room (laminar flow)	×1
MDT Boardrooms (video-linked)	×4

Anchor clinicians: programme builders driving outcomes, research, and referral growth

Adult haemato-oncology & BMT lead

BMT/CAR-T/acute leukaemia · leads haem-onco franchise, anchors transplant ID partnerships

Paediatric haemato-oncology & BMT lead

Paediatric HSCT, thalassemia, PID · opens national referral network for paediatric BMT

Transplant hepatology & liver surgery

Liver transplant · hepatobiliary · high-ticket, high-complexity — signature VPS capability

Kidney transplant pair (nephrology + urology)

Live and deceased kidney transplant · uses dialysis, ICU and OT assets · compounds transplant brand

Solid tumour surgical oncology lead

Complex resections · sarcoma · neuro-oncology · anchors MDT culture and tumour-board referral engine for NCR

Molecular pathology & genomics head

NGS · pharmacogenomics · biomarkers · builds diagnostic differentiator and trial eligibility pipeline

Rare disease & immunology lead

LSDs · PID · metabolic paediatrics · creates national second-opinion franchise for rare cohorts

Neuromuscular programme lead

SMA · muscular dystrophy · gene therapy · low volume, high complexity — brand differentiator

ICU / transplant infectious disease lead

Immunocompromised host pathways · clinical safety across BMT, transplant and AIIR programmes

RECRUITMENT PROPOSITION · Institute leadership roles · protected BMT and transplant infrastructure · dedicated research budget · outcomes registry · dual-city practice bridge with VPS Lakeshore Kochi

Four patient engines. Pre-launch commitments are required from each channel before clinical opening

A — NCR Consultant Referrals

25–40 named referring consultants signed before opening. NCR oncologists, haematologists, nephrologists, neurologists and paediatricians operating on rapid-access tumour board slots, shared-care protocols, and structured outcomes feedback loops.

Pre-launch target: 25–40 named consultants

B — Hospital Spoke Partnerships

Formal compacts with secondary NCR hospitals that diagnose but cannot execute BMT, CAR-T, complex transplant or genomic work-ups. VPS-supervised escalation protocols define the referral interface and case-handover protocol.

Pre-launch target: 8–12 hospital spokes

C — VPS Lakeshore Network Referrals

Structured referral engine from VPS Lakeshore Kochi and future network nodes. Quaternary cases are routed to Delhi for capabilities not available in spoke geographies. Delhi PMC joins existing MDT cadence from Day 1.

Lakeshore Kochi + future network nodes

D — Medical Value Travel

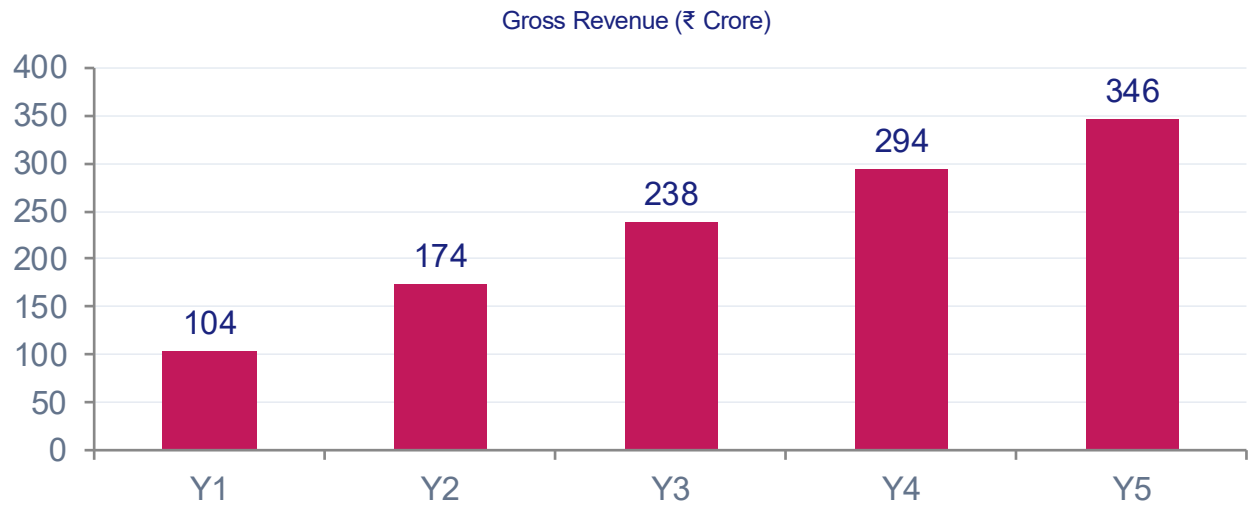
Dedicated international patient desk with concierge services, visa and travel coordination, language support, and payer and scheme navigation. Primary catchment: SAARC, Africa and GCC. Aspirational Prevention Clinic provides a well-patient entry point.

SAARC · Africa · GCC

FEEDER PATHWAYS: Hereditary cancer clinic & genetic counselling · Thalassemia/aplastic anaemia work-up · Pre-transplant preparation (liver, kidney, BMT) · Precision prevention & liquid biopsy · National second-opinion pathway

Feeder pathways within each vertical capture lower-acuity intake — building the complex-episode pipeline without diluting institutional focus.

Operating break-even at Year 3. Mature economics at Year 5



Year	Occupancy	ARPOB (₹)	Revenue (₹ Cr)	EBITDA %
Y1	35%	60,000	104	-5% to 0%
Y2	50%	70,000	174	8%
Y3	**60%**	**80,000**	**238**	**16%**
Y4	68%	87,000	294	21%
Y5	**75%**	**93,000**	**346**	**24%**

CONDITIONS FOR PLAN DELIVERY

Phased capex

LINAC 2 and optional heart/lung capability deferred until threshold volumes are demonstrated. Phasing is a financial control, not a design concession.

Concentrated doctor cost

Anchor programme leads funded at the required level — cost not spread across generic specialties outside the five verticals.

Case-mix discipline maintained

At least 65–70% of revenue from the five defined verticals by Year 5. Tracked monthly against CMO-chaired case-mix review.

Project cost envelope

Planning assumption: ₹550–650 crore excluding land (shell, MEP, medical equipment, IT and pre-operative). Returns compress above this envelope without sustained ARPOB outperformance.

Indicative projections. Excludes land cost. Benchmarked against Max Healthcare, Apollo Hospitals and Narayana Health comparable operator data.

NEXT STEPS

Path Forward to establish Delhi PMC among India's most credible hubs for precision-led complex care

1. **Issue architect brief** to qualified healthcare design practices
2. **Finalise site and statutory approvals** for Delhi PMC
3. **Open anchor clinician conversations** across the five program verticals
4. **Pre-sign the NCR referral network** with 25–40 consultants and 8–12 hospital spokes
5. **Lock the Kochi proof-of-concept cadence** on milestones, outcomes metrics, and dual-city bridge design